# The Complete Guide to Pelvic Floor Care During Pregnancy

## Understanding, Training & Recovery — A Comprehensive Resource

**Based on workshop by:** Sheetal Anand

Pre & Postnatal Expert

Prepared for: HMT × Family

March 2026

## Contents

**Part 1:** Understanding Your Pelvic Floor

**Part 2:** The Breathing Connection

**Part 3:** Trimester-by-Trimester Guide

**Part 4:** The Kegels Myth — Critical Reading

**Part 5:** Warning Signs & When to Seek Help

**Part 6:** Daily Care Practices

**Part 7:** Postpartum Recovery

**Part 8:** Exercises Reference

**Part 9:** Q&A from the Workshop

**Part 10:** Quick Reference Summary

**THE #1 INSIGHT**

**Your pelvic floor isn't just about being "strong" — it needs to be *flexible, controlled, and dynamic*. Like a hammock that can hold weight AND spring back.**

# Part 1: Understanding Your Pelvic Floor

## What Is the Pelvic Floor?

Your pelvic floor is a group of muscles and connective tissues that form a **hammock-like structure** at the base of your pelvis. Understanding this anatomy is the first step to caring for it properly.

### The Bones: Pelvic Girdle

- Bowl-shaped bony structure covering from the sides
- The hard part you feel at hip level where you wear your lowers
- Includes your "sit bones" — what you actually sit on

### The Muscles: Pelvic Floor

- Stretches from your pubic bone (front) to your tailbone (back)
- Connects one sit bone to the other (side to side)
- Forms a supportive "hammock" for all your pelvic organs

### The Hammock Visualization

Imagine a hammock strung between two trees. When you lie in it, it sinks to support your weight. When you get up, it springs back to neutral.

**Your pelvic floor works exactly the same way.**

It must be able to BOTH stretch under pressure AND return to its resting state.

## What Does the Pelvic Floor Do?

| Function | Description |
|---|---|
| **Organ Support** | Holds up your bladder, uterus, and rectum — like a shelf for your organs |
| **Bladder Control** | Controls when you urinate — holding and releasing |
| **Bowel Control** | Controls bowel movements and passing gas |
| **Spinal Stability** | Attached to tailbone; helps stabilize your spine and pelvis |
| **Sexual Function** | Plays a role in sexual sensation and function |
| **Pressure Management** | Works with diaphragm and core to manage internal pressure |

## Why It Matters During Pregnancy

Your uterus sits *directly* on this hammock of muscles. As your baby grows:

- Weight on pelvic floor increases **dramatically** — imagine adding more and more weight to a hammock
- Muscles must adapt constantly to support this changing load
- Untrained muscles lead to problems during pregnancy, labor, AND recovery

**⚠ THE CORE PROBLEM**

An untrained pelvic floor can cause: urinary leakage, pelvic pain, difficult labor, prolonged recovery, and long-term incontinence. Training it properly now prevents these issues later.

# Part 2: The Breathing Connection

**THIS IS THE INSIGHT MOST WOMEN MISS**

**Your breath directly moves your pelvic floor. Every deep breath is a mini pelvic floor exercise — no Kegels required.**

## How Breathing Moves Your Pelvic Floor

| When You... | Your Diaphragm... | Your Pelvic Floor... |
|---|---|---|
| **Inhale** | Moves DOWN | Stretches DOWN (like someone lying in the hammock) |
| **Exhale** | Moves UP | Returns UP (like hammock springing back) |

The diaphragm (breathing muscle under your lungs) and pelvic floor are connected. They move together in rhythm with every breath. This is why **conscious breathing is one of the most effective ways to train your pelvic floor**.

## Diaphragmatic Breathing Practice

**DAILY PRACTICE (2-3 MINUTES)**

1. **Sit straight** — spine tall, shoulders relaxed, not hunched
2. **Place hands on sides of ribs** — at chest level, not waist
3. **Inhale slowly through nose** — feel your ribs expand sideways, like a rubber ring inflating
4. **Feel expansion in all directions** — front, sides, AND back
5. **Exhale slowly** — feel everything return naturally
6. **Repeat** — 10-15 breaths, 2-3 times daily

**Why this works:** When your ribs expand and diaphragm descends, the pressure travels down to your pelvis, gently stretching and then releasing your pelvic floor muscles. This natural movement trains flexibility without any "exercise" needed.

# Part 3: Trimester-by-Trimester Guide

What your pelvic floor needs changes dramatically throughout pregnancy. What helps at 12 weeks may actually hurt at 38 weeks. Here's what to focus on in each phase:

## First Trimester (Weeks 1-13)

**Goal: BUILD SUPPORT as weight starts increasing**

In early pregnancy, you want to strengthen the hammock. The uterus is growing, and you need your pelvic floor to provide solid support.

### Do

- Kegels (tighten-hold-release)
- Core engagement exercises
- Diaphragmatic breathing
- General fitness activities

### Avoid

- Nothing specific to avoid yet
- Listen to your body
- Avoid if any complications

## Second Trimester (Weeks 14-27)

**Goal: MAINTAIN & PROTECT — Don't let baby descend too early**

The baby is growing significantly now. You want to maintain pelvic floor support to prevent premature descent. This is NOT the time to over-loosen.

### Do

- Moderate Kegels (once daily or few times weekly)
- Dynamic movements (not static holds)
- Continue breathing practice
- Gentle hip mobility

### Avoid

- Holding deep squats for long periods
- Aggressive daily Kegels
- Over-loosening exercises
- Prolonged malasana holds

## Third Trimester (Weeks 28-40, especially 35+)

**Goal: PREPARE TO RELEASE — Get ready for baby's descent and labor**

This is the critical shift. Your baby needs to descend, and your pelvic floor needs to LOOSEN to allow this. A tight pelvic floor at this stage will make labor harder.

### Do

- Hip-opening exercises
- Supported squats (malasana) — held longer now
- Side lunges, elevated lunges
- Relaxation & jaw loosening
- Perineal massage

### Avoid

- Frequent Kegels (you need loosening!)
- Exercises focused on "holding" or "tightening"
- Anything that makes pelvic floor MORE tense

# Part 4: The Kegels Myth — CRITICAL READING

**THIS SECTION COULD CHANGE YOUR BIRTH EXPERIENCE**

Most women hear "do Kegels" and do them constantly from first trimester until delivery day. **This is a mistake.**

## What Are Kegels?

Kegels are exercises that involve tightening and releasing your pelvic floor muscles. The sensation is the same as:

- Holding your pee midstream
- Then letting go

That's it. Tighten, hold briefly, release. Simple.

## The Problem: Kegels Are NOT Always Beneficial

| Situation | Kegels? | Why |
|---|---|---|
| First trimester | **Yes** | Building support as weight increases |
| Second trimester | **Moderate** | Maintaining without overdoing |
| 35+ weeks | ⚠ **Reduce significantly** | You need LOOSENING for baby's descent |
| Already tight pelvic floor | **No** | Makes the problem worse |
| During labor | **No** | You need to PUSH, not hold |

## What Happens If You Overdo Kegels?

- Pelvic floor becomes **too tight**
- Baby can't descend easily into birth canal

- Pushing during labor becomes much harder
- **Increased risk of tearing**
- Longer, more exhausting labor

> **THE RIGHT APPROACH**
>
> **Think of Kegels as ONE tool, not THE tool. Balance with breathing exercises, hip openers, relaxation, and massage. Adapt your practice to your trimester.**

## Remember: Your Goal is CONTROL, Not Just Strength

A well-trained pelvic floor can:

- Tighten when you need support
- Loosen when you need to release
- Respond to your conscious commands
- Work in rhythm with your breath

Constant Kegels only train ONE of these abilities. You need all of them for a smooth labor and recovery.

# Part 5: Warning Signs & When to Seek Help

## Signs Your Pelvic Floor May Be Too Weak

| Symptom | What It Means |
|---|---|
| **Urinary incontinence** | Can't hold pee when you laugh, sneeze, or cough — loss of muscle control |
| **Heaviness or dragging feeling** | Sensation of weight or pressure in pelvis — organs not fully supported |
| **Severe low back/hip pain** | Beyond normal pregnancy discomfort — spinal stabilization affected |
| **Constipation** | Despite good diet and movement — bowel control affected |
| **Difficulty with simple movements** | Walking, sitting, standing become hard — muscles not supporting daily activities |
| **Difficulty controlling bowels** | Gas or bowel incontinence — pelvic floor control compromised |

**SEEK PROFESSIONAL HELP IMMEDIATELY IF YOU EXPERIENCE**

- **Persistent leakage** that doesn't improve with basic exercises
- **Severe pelvic pain** that restricts your daily movement
- **Sensation of something "falling"** inside — like organs descending
- **Visible bulge** in vaginal area
- **Painful postpartum recovery** beyond expected discomfort

**Early help = faster recovery. Don't wait to address these symptoms.**

## Signs Your Pelvic Floor May Be Too Tight

- Pain during intimacy

- Difficulty fully emptying bladder
- Chronic pelvic tension or pain
- Unable to relax the area even when trying

# Part 6: Daily Care Practices

## Essential Daily Practices

| Practice | How Often | Why It Helps |
|---|---|---|
| **Diaphragmatic breathing** | 2-3 minutes, 2-3× daily | Natural pelvic floor training through breath rhythm |
| **Good posture** | Continuously | Reduces unnecessary strain on pelvic floor |
| **Soft seating surface** | When sitting long periods | Hard surfaces create extra pressure |
| **Jaw relaxation** | Throughout day, especially when stressed | Relaxed jaw signals whole body to release tension |
| **Gentle movement** | Daily | Keeps blood flowing, muscles responsive |
| **Perineal massage** | 3rd trimester, as comfortable | Improves flexibility of tissues for birth |

## The Jaw-Pelvis Connection

This may seem strange, but it's scientifically backed: **your jaw and pelvic floor are neurologically connected**.

- When you clench your jaw, your whole body tenses — including pelvic floor
- When you relax your jaw, your nervous system signals ALL muscles to relax
- During labor, keeping jaw loose helps pelvic floor stay open

**HORSE BREATHING (LABOR PREP)**

Relax your jaw completely, let lips flap loosely like a horse's lips while exhaling. This sounds silly but is highly effective at releasing pelvic tension. Practice regularly in third trimester.

## What NOT to Do

| Habit to Avoid | Why It's Harmful |
|---|---|
| Constantly sucking belly in | Creates chronic tightness in entire core-pelvic system |
| Aggressive Kegels all day | Overtightens pelvic floor; counterproductive near labor |
| Holding urine frequently | Strains muscles and can lead to infections |
| Ignoring warning signs | Small problems become big problems if not addressed |
| Intense exercise without guidance | Risk of harm without proper form and trimester-appropriate modifications |

# Part 7: Postpartum Recovery

## What Happens During Delivery

Understanding what your pelvic floor goes through helps you recover better:

- **Vaginal delivery:** Pelvic floor stretches significantly to allow baby through. Muscles are temporarily very loose and may feel "absent."
- **C-section:** Less direct trauma to pelvic floor, but still affected by pregnancy weight and hormones.

In both cases, the "hammock" is still there — it's just stretched and needs time to recover.

## Recovery Timeline

| Phase | Focus | What to Do |
|---|---|---|
| **Weeks 1-2** | Rest & Heal | Only gentle breathing. Let your body recover. No exercise. |
| **Weeks 2-6** | Gentle Activation | Gentle breathing continues. Very light movements only if comfortable. |
| **After 6 weeks** (vaginal delivery) | Gradual Rebuilding | Begin postnatal yoga. Gentle core activation. Get doctor clearance first. |
| **3-6 months** (C-section) | Later Start | Core and pelvic floor work begins later. Always get doctor's approval. |

**⚠ WARNING: DON'T RUSH CORE EXERCISES**

Overdoing core work postpartum can cause **diastasis recti** — a separation of the abdominal muscles that creates a persistent belly bulge ("mummy tummy"). This is difficult to reverse once established.

Gentle, guided progression is essential. More is NOT better in early postpartum.

## Signs of Good Recovery

- Gradually regaining bladder control
- Decreasing heaviness sensation
- Able to do light activities without discomfort
- Pelvic floor responds to Kegels (can feel the squeeze)

## Signs to Seek Help

- Persistent incontinence beyond 6 weeks
- Pain during basic movements
- No improvement despite gentle exercises
- Feeling of "something falling out"

# Part 8: Exercises Reference

## All Trimesters — Foundation Exercises

| Exercise | How to Do It | Benefits |
|---|---|---|
| **Diaphragmatic Breathing** | Sit tall, hands on ribs, inhale to expand sides, exhale to release | Natural pelvic floor training, stress relief |
| **Cat-Cow** | On hands and knees, alternate arching and rounding spine | Spine mobility, gentle core activation |
| **Pelvic Tilts** | Lying or seated, tilt pelvis forward and back gently | Lower back relief, pelvic awareness |
| **Hip Circles** | Standing, make gentle circles with hips | Hip mobility, blood circulation |

## Third Trimester — Labor Prep Exercises

| Exercise | How to Do It | Benefits |
|---|---|---|
| **Supported Malasana** | Deep squat with support (wall, blocks, partner) | Opens pelvis, lengthens pelvic floor |
| **Side Lunges** | Wide stance, shift weight side to side | Hip opening, inner thigh stretch |
| **Elevated Lunges** | One foot elevated, gentle lunge | Hip flexor release, pelvic mobility |
| **Butterfly Pose** | Seated, soles of feet together, knees out | Hip opening, groin stretch |
| **Perineal Massage** | Gentle massage of perineal tissues | Tissue flexibility, tear prevention |

## Postpartum — Gentle Recovery

| Exercise | When to Start | Benefits |
|---|---|---|
| **Breathing Only** | Day 1 | Gentle activation, no strain |
| **Gentle Kegels** | After 2-4 weeks (if comfortable) | Reconnect with pelvic floor |
| **Postnatal Yoga** | After 6 weeks (with clearance) | Full body recovery, guided progression |

# Part 9: Q&A from the Workshop

Real questions from participants, answered by Sheetal Anand:

### Q: Can Kegels cause cervix to open too early?

**A:** No, if practiced under professional guidance with trimester-appropriate intensity. The exercises are designed to support, not harm. However, those with specific complications should always consult their doctor first.

### Q: How does jaw loosening help pelvic floor?

**A:** When you loosen your jaw, your nervous system sends a signal to your ENTIRE body to relax — including pelvic floor muscles. They're neurologically connected. Keeping jaw loose during labor helps the pelvic floor stay open for baby's descent.

### Q: What about pelvic pain at night while sleeping?

**A:** Try gentle hip rotations before sleep to activate blood circulation. Standing too long during the day can reduce blood supply, causing pain at rest. A little movement before bed can help significantly.

### Q: How does a birthing ball help?

**A:** It provides a softer surface than hard chairs, reducing pressure on the pelvic floor. Sitting on hard surfaces for extended periods puts extra strain on these muscles.

### Q: What if I have severe pelvic bone pain when lying down or sitting up?

**A:** This requires personalized guidance. Severe pain that restricts movement is a sign to consult a professional. General advice may not apply to specific complications.

### Q: Can women with C-sections do pelvic floor exercises?

**A:** Yes, but generally starting 3-6 months postpartum, with doctor's approval. The timeline is longer because core healing takes priority after abdominal surgery.

### Q: I feel sharp pelvic pain during cat-cow. What should I do?

**A:** Reduce the intensity — take the movement smaller and gentler. Try perineal massage to ease tension. If pain persists, consult a prenatal specialist for modifications.

# Part 10: Quick Reference Summary

## The 5 Core Principles

1. **Dynamic > Static** — Train for flexibility, not just strength
2. **Breath is everything** — Natural breathing trains pelvic floor automatically
3. **Trimester matters** — What helps at 12 weeks may hurt at 38 weeks
4. **Listen to your body** — Awareness is your best tool
5. **Care now = easier later** — Investment now pays off in labor and recovery

## The 3 States of Pelvic Floor

| State | What It Means | The Problem |
|---|---|---|
| **Too Tight** | Hammock pulled taut, no give | May tear during labor; blocks baby's descent |
| **Too Loose** | Hammock sags to ground | Organs unsupported; incontinence; heaviness |
| **Just Right** | Flexible, controlled, responsive | No problem — can tighten AND loosen as needed |

## Trimester Cheat Sheet

| Trimester | Goal | Kegels? | Focus |
|---|---|---|---|
| **1st** | Build support | Yes | Strengthening, breathing |
| **2nd** | Maintain & protect | Moderate | Dynamic movement, no over-loosening |
| **3rd (35+)** | Prepare to release | ⚠ Reduce | Hip openers, relaxation, perineal massage |

**THE BOTTOM LINE**

**Your pelvic floor is the foundation supporting your entire pregnancy journey. Treat it like the dynamic, responsive system it is — not just a muscle to "tighten."**

**The goal isn't strength. It's CONTROL.**

---

**Based on workshop by Sheetal Anand, Pre & Postnatal Expert**

Transcribed and structured by HuMT

For the HMT × Family Group • March 2026

⚠ This guide is educational. Always consult your healthcare provider for personalized advice, especially if you have complications.